Anaphylaxis

Call ambulance

Check	
Airway	**open airway** (consider intubation if needed)
Breathing	assess effectiveness of ventilation + give **O2 10L/min (aiming for > 94%)**
Circulation	check pulse, BP and perfusion, **fix large bore IV cannula**
Disability	assess consciousness level by **GCS**
Exposure	**check skin changes** (urticarial), insect bite site, facial or lip **swelling (angioedema)**

Start fluids 0.9% saline for:

- Adults: **500ml-1L in 5-10 min**
- Children: **20 ml/kg**

There are no absolute contraindications to Epinephrine in the setting of anaphylaxis

- **Adrenaline IM 1:1000 is the first and main treatment for anaphylaxis** → Anterolateral aspect – middle 1/3 of the thigh Repeat after 5 min if no improvement..
  - If patient is on Beta blocker and no response to Adrenaline → **Give Glucagon** (1-5 mg slowly IV bolus over 5 min)
  - If bronchospasm not responsive to Adrenaline → **Give inhaled Salbutamol**
- **Non-Sedation Oral Antihistamine** chlorphenamine (piriton) after stabilizing patient especially if presented with skin symptoms (urticaria and or angioedema).
- **Antihistamines & Hydrocortisone are consideration Adjunctive agents.**

Age	IM Adrenaline (1:1000)	IM or slow IV Hydrocortisone	IM or slow IV Chlorphenamine (Piriton)
> 12 Yr.	0.5 ml	200 mg	10 mg
6 – 12 Yr.	0.3 ml	100 mg	5 mg
6 Mon – 6 Yr.	0.15 ml	50 mg	2.5 mg
< 6 Mon	0.1 - 0.15 ml	25 mg	250 mcg/kg